GP-STAR

ST1530-1 · ^{68}Ga-GP-01 FAP PET · Phase I/II

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Protocol No: ST1530-1
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CLINICAL PROTOCOL

Protocol Title: An Exploratory, Open-label Study of Safety, Pharmacokinetics, Biodistribution and Dosimetry of ^{68}Ga-GP-01 in Participants with Solid Tumors (GP-STAR)
Protocol Number: ST1530-1
Compound: ^{68}Ga-GP-01
Study Phase: Phase I/II
Sponsor: Grand Sirtex (Chengdu) Medical Technology Co., Ltd.

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What this study is, in one breath

Give a person with solid tumors one FAP PET injection, take pictures, then compare them to surgery or biopsy plus FAP staining.

Diagnostic imaging study. Not a cancer treatment trial.

> **If the protocol feels like a wall of words**
>
> A protocol is a recipe: who can join, what is injected, how much, when you scan or treat, and how you decide if a picture or outcome was right. This sheet is the same idea in normal English. It does not replace the signed site copy.

1. The tracer

Gallium-68 GP-01 is a small-molecule FAP inhibitor for PET/CT. FAP sits on cancer-associated fibroblasts (stroma), not usually on the epithelial cancer cell.

- Ga-68 is a positron emitter (T½ 68 min → 511 keV pair). Not a gamma emitter.
- Dose: 185 MBq ± 10% (5 mCi ± 10%), mass ≤ 50 µg, one IV bolus.
- Strong P-gp / BCRP inhibitors are prohibited (1 week or 5 half-lives).

2. Two parallel cohorts (~10 each)

	Cohort 1	Cohort 2
Who	Solid tumors with suspected peritoneal mets	

(gastric, CRC, ovarian/FT and others).
Path-proven primary.

NSCLC with suspected nodal mets, heading to

dissection or biopsy.

Truth	Surgery/laparoscopy + path + FAP IHC	Node path + FAP IHC

FDG PET/CT within 60 days before GP-01. Do not change the operation off this scan. First 6 people get multi-timepoint PET, PK, biodistribution, and dosimetry; everyone else gets one PET (~30–60 min, may move).

3. How a scan is scored

Pathology of the piece they actually took is truth. Hot PET + negative path on that resected region = false positive (v1.2 fixed this). Sensitivity/specificity/PPV/NPV/accuracy are calculated separately per cohort. Compare FAP vs FDG on SUVmax, SUVmean, TBR.

Primary endpoints

Safety (TEAEs, labs, ECG; CTCAE v6.0) and preliminary diagnostic performance vs pathology, plus SUV comparison with FDG.

What to know cold

Q. What is GP-01?
A. Small-molecule FAP inhibitor labeled with Ga-68 for PET/CT.

Q. Dose?
A. 185 MBq ± 10% (5 mCi ± 10%), mass ≤ 50 µg, one IV bolus.

Q. Two cohorts?
A. Peritoneal mets of solid tumors, and NSCLC nodal mets. ~10 each.

Q. Truth?
A. Pathology + FAP IHC. Hot PET + negative resected path = FP.

Q. Change surgery off this scan?
A. No. Conventional imaging remains the clinical driver.

How to use this with Board Studio

Open the matching protocol module and press play. The left pane is the signed page. Use this PDF when legalese stalls you. If anything conflicts, the signed protocol wins.

Disclaimer. Educational summary for board-style study. Not medical advice, not a recruitment brochure, not a substitute for the signed protocol, and not affiliated with the sponsor, ABR, ABNM, ACGME, SNMMI, NRC, or FDA. Confirm numbers against the current site copy before any clinical use.